

Table of Contents

01.10	Introduction.....	2
01.20	POLICIES	3
01.21	Intake	3
01.22	Financial Eligibility.....	3
01.23	Assessment.....	3
01.23.10	Timeliness Standards.....	3
01.23.20	Consent and Authorized Representative.....	3
01.23.30	Participant Service Choice form.....	4
01.23.40	Assessment	4
01.24	Medical Eligibility Criteria	4
01.30	Case Management.....	8
01.31	Initial Visit	8
01.32	Service Plan.....	8
01.33	Service Authorization.....	8
01.34	Monitoring and Re-evaluation	11
01.35	Documentation	12
01.36	Termination.....	12

01.10 Introduction

The Community Long Term Care Human Immunodeficiency Virus (HIV)/Acquired Immune Deficiency Syndrome (AIDS) Medicaid waiver assists persons who have been diagnosed as HIV positive or having AIDS. This program helps persons stay at home as long as possible and avoid extended hospitalization.

This policy contains information specific to the HIV/AIDS Waiver. Otherwise, policy is to be followed as outlined in the CLTC Community Choices Policy and Procedure Manual.

01.20 POLICIES

01.21 Intake

Follow Policy as outlined in the CLTC Community Choices Manual (1.2). Note: Residents of a Community Residential Care Facility are eligible to participate in the HIV/AIDS waiver but with certain service restrictions.

The policies relating to confidentiality are the same as for the Community Choice waiver.

The following criteria must be met in order for a referral to be considered appropriate:

1. Applicant must be currently located in S.C. or intend to locate in S.C.; and,
2. Applicant must have been diagnosed as being HIV positive or having AIDS.

Note: There is no age requirement or need for functional dependency to meet intake criteria.

01.22 Financial Eligibility

Financial eligibility (Medicaid) must be established before a participant can be enrolled in the waiver. The same procedures utilized in the Community Choices waiver will be used to establish/verify Medicaid eligibility.

01.23 Assessment

01.23.10 Timeliness Standards

An applicant for this waiver must be visited **within ten (10) business days** from case assignment.

01.23.20 Consent and Authorized Representative

The Consent Form must be signed by the participant or authorized representative at the time of the assessment.

Please refer to the Community Choices Manual and follow policy regarding the appointment of an Authorized Representative (Chapter 2 2.22.01).

Note: If the participant is in the custody of DSS, the DSS case worker may sign the Consent Form.

01.23.30 Participant Service Choice form

The CLTC Participant Service Choice should be signed **at the time of assessment** indicating the participant's desire regarding program participation and location choice.

Note: If the participant is in the custody of DSS, the DSS case worker may sign the CLTC Participant Service Choice Form.

01.23.40 Assessment

The Assessment will be completed by the **SCDHHS Worker**. The HIV Physician's Information Form must be sent to the physician, physician assistant, or nurse practitioner for completion. This includes a recommendation for at-risk for hospitalization and qualified signature. Receipt of the completed HIV Physician's Information Form marks a completed assessment. Timeliness standard for level of care determination is three **(3) business days** upon the receipt of a completed assessment. A tentative level of care is not required while awaiting the receipt of the HIV Physician's Information Form

The Assessment date **MUST** be within 30 calendar days of entry into the waiver.

If the Assessment is older than 30 calendar days, a new Assessment must be completed and a level of care must be determined and entered into Phoenix.

The HIV Physician's Information Form is valid for 180 days from the physician, nurse practitioner or physician assistant's signature.

HIV Physician's Information Form should be scanned into Phoenix under the Assessment Scan Tag.

01.24 Medical Eligibility Criteria

The CLTC HIV/AIDS waiver program utilizes only one level of care - at risk for hospitalization. The level of care criteria is outlined below. If the participant is seeking admission to a nursing facility or other long term care programs such as Administrative Days, then the participant must meet the established level of care for those programs which is skilled or intermediate nursing facility level of care.

When the assessment is completed, including the information required from the physician or physician's office, a LOC will be recommended. If the LOC recommended is Medically Ineligible based on lack of associated HIV diagnosis or CD4 count over 500, the assessor may request a LOC exception. This information will be sent to Central Office Designee via Phoenix. The approval or

denial will be returned to the assessor via Phoenix.

If a new assessment is required for waiver enrollment due to the previous assessment being over thirty (30) days old, and Central Office Level of Care Exception Approval was received following the previous assessment, another Central Office Level of Care Exception approval is not required prior to enrollment. For example, an applicant's HIV assessment was completed May 1, 2014 and a level of care exception was granted on May 15, 2014. The applicant requires another assessment for an enrollment date of June 5, 2014. Another Central Office level of care exception Approval is not required.

Criteria for a participant age 6 years and older

Have one of the following diagnoses:

Diagnosed as having AIDS which includes but is not limited to the following:

AIDS defining illnesses such as:

- Candidiasis of the esophagus, trachea, bronchi and lungs;
- Cervical cancer, invasive;
- Coccidioidomycosis, disseminated or extrapulmonary;
- Cryptococcus;
- Cryptosporidiosis;
- Cytomegalovirus (CMV) of an organ other than the liver, spleen, or lymph nodes;
- Cytomegalovirus retinitis (with loss of vision);
- Herpes Simplex, chronic ulcers;
- Histoplasmosis, disseminated or extrapulmonary;
- HIV encephalopathy (dementia);
- Isosporiasis, chronic intestinal;
- Kaposi's sarcoma (KS);
- Lymphoma of the brain;
- Lymphoma, Burkitts or immuroblastic (or equivalent term);
- Mycobacterium avium complex (MAC) or Mycobacterium avium infiltrate (MAI);
- Mycobacterium TB
- M. kansasii disease;
- Pneumocystis carinii pneumonia (PCP);
- Recurrent Pneumonias;
- Progressive multifocal leukoencephalopathy;
- Salmonella septicemia, recurrent;
- Toxoplasmosis of the brain;
- Wasting syndrome caused by HIV.

or

Diagnosed as being HIV positive **and** have had two or more episodes of specific related conditions which include, but are not limited to the following:

Anemia;
Granulocytopenia;
Thrombocytopenia;
Documented fever;
Weight loss;
Oral candidiasis;
Oral hairy leukoplakia;
Herpes zoster;
Diarrhea;
Persistent dermatitis;
Recurring vaginal infections;
Fatigue;
Generalized lymphadenopathy;
Night sweats;
Leukopenia;
Marked restriction of activities of daily living; and,
Deficiencies of concentration.

and

Have a CD4 level equal to or less than 500

and

The physician, physician assistant, or nurse practitioner must determine that the participant is at risk for hospitalization in the near future.

Criteria for a participant from birth to 6 years:

1. Must be HIV positive;

and

2. Should have a declining CD4 level;

and

3. Must have had at least one of the clinical manifestations common in children, which include, but are not limited to the following:

Fever;
Weight loss;
Thrush;
Lymphadenopathy;
Diarrhea;

Otitis media;
Eczematoid skin rash;
Candida skin infections (diaper rash);
Parotitis or salivary gland enlargement;
Invasive or disseminated infections; Malaise;
Recurrent upper respiratory infections (URIs);
Failure to thrive;
Developmental delay;
Low birth weight;
Loss of attained milestones;
Microcephaly;
Other common childhood infections;

and

4. The physician, physician assistant, or nurse practitioner must determine that the child is at risk for hospitalization in the near future.

01.30 **Case Management**

The same procedures established in the Community Choice Waiver will be used to conduct case management. Participants may continue to reside in the CRCF and receive restricted services through the waiver.

The participants may receive the following services through the waiver while residing in the CRCF:

- Case management
- Nutritional supplements

Participant may NOT receive any other services through the waiver, including PCI, PCII, PDN, home delivered meals, environmental modifications, and/or attendant.

NOTE: Children enrolled in the Medicaid hospice benefit and HIV waiver may receive services through the waiver; however, Case Manager must seek Central Office approval.

01.31 **Initial Visit**

The **Case Manager** must complete an initial visit within thirty (30) days after enrollment. Refer to the initial visit requirements outlined in the CLTC Community Choices Manual (05.31).

01.32 **Service Plan**

The same procedures established in the CLTC Community Choices Manual (6.2) will be used to develop and implement a service plan.

01.33 **Service Authorization**

The **Case Manager** will authorize waived services for the participant based on needs. As in the Community Choices waiver, participant choice will be used to select a provider. (Refer to Section 07.22 in Community Choice Policy Manual.) Waivered services are provided by contract/enrolled Medicaid providers to an eligible participant and include:

Attendant Care
Companion
Case Management (CM)
Personal Care I services (PCI)
Personal Care II services (PCII)
Home Delivered meals (HDM)
Home Accessibility Adaptations - Environmental Modification

Private Duty Nursing (Nursing services)
Pest Control
Advanced Pest Control
Specialized Medical Equipment (Handheld Shower and Nutritional Supplements)

Note: A participant can be enrolled in a waiver and on Hospice at the same time but is limited in the services that may be authorized. (See Hospice Section.)

The only waiver services requiring a Physician's Order are Medicaid Nursing Services and Nutritional Supplements.

Case Management, Companion, Personal Care I and II Services, Home-Delivered Meals, Home Accessibility Adaptations - Environmental Modification, Pest Control, Advanced Pest Control, Specialized Medical Equipment, and Attendant Care are authorized following procedures established in the Community Choices waiver.

Medicaid Nursing Service and Nutritional Supplements are authorized as follows:

Medicaid Nursing Service

Medicaid Nursing Service does require a Physician's Order. To authorize nursing service, the **Case Manager** contacts the participant's physician concerning the participant needs, availability of skilled nursing services, and projected service schedule, including amount, frequency and anticipated duration of services. The initial CLTC PDN Physician's Order for nursing service is obtained at this time. Contact is made with the provider and service schedule and start date are negotiated prior to authorization of the service in Phoenix. The provider will evaluate the participant, obtain skilled nursing orders from the physician, and send the case manager a written statement including treatment goals and any recommendations for changes in the service schedule. The treatment plan submitted by the provider will be attached to the Service Plan as an addendum and will serve as the problem/goal/intervention for that particular component. The **Case Manager** and provider should negotiate an effective solution to any differences that may arise between the projected and the final service schedules.

Situations which may indicate a need for nursing services include but are not limited to:

- IV or IM drug therapy;
- wound/skin care;
- monitoring of an unstable condition with treatments; and/or,
- personal care which in professional judgment exceeds the capabilities of Personal Care II.

Steps for the Case Manager:

- Determines need for professional intervention with participant and/or authorized representative. Discusses service needs with participant and/or authorized representative. Informs participant and/or authorized representative of providers. Explains procedures, service options, and the responsibilities of each party involved to participant and/or authorized representative.
- Contacts physician and forwards copy of PDN Physician's Order to physician for signature. (SCDHHS Worker may have already obtained the Physician's Order to avoid service delay.)

Note: In an emergency situation, a physician's verbal order may be obtained by a **Nurse Consultant**. A signed Physician's Order must be secured **within two (2) calendar days**.

- Creates a referral for PDN in Phoenix to provider of choice.
- After acceptance negotiates date for evaluation visit. Instructs provider on the information needed from this evaluation: problems, goals, and interventions and frequency.
- **Case Manager** completes Service Authorization for evaluation visit including an end date and forwards Service Authorization along with copy of the physician's order and service plan to provider. **Do not proceed with any further authorizations until summary of evaluation visit has been received.** Activities must be included in the Narrative.
- Receives written statement (treatment plan) from provider which includes treatment goals and any recommended service schedule changes and attaches treatment plan from provider to service plan.
- Negotiates schedule of nursing services with provider.
- Completes Service Authorization including units of service per week.
- Updates service plan to include nursing services schedule.
- Documents activities in the Narrative.

Nutritional Supplements

The participant must choose an enrolled provider listed on the Provider Choice Form.

A physician's order is required prior to authorization of nutritional supplements.

A maximum of two (2) cases of nutritional supplements can be authorized per month. If a participant is being tube fed, this service should not be authorized.

This service is to be addressed in the Service Plan but a copy of the Service Plan should not be sent to the provider. The service is to be authorized in Phoenix. If the service needs to be changed, the current Authorization would need to be terminated and a new Authorization issued.

Since Nutritional Supplements are also available as a hospice service, this waiver service should not be authorized for a waiver participant who is also enrolled in hospice. If the supplies are available through another source (such as home health, hospice, or community resources), these sources should be used prior to authorizing Medicaid services.

Steps for the Case Manager:

1. Identifies need for Nutritional Supplements.
2. Obtains CLTC Physician's Order for Nutritional Supplements Form.
3. Asks the participant to choose a provider from the Provider Choice Form.
4. Makes a referral to the Provider in Phoenix.
5. Authorizes the services by selecting the appropriate service in Phoenix.

Note: No end date is necessary; the schedule/frequency line must be completed.

6. Sends appropriate form to provider. Copy of Service Plan does not need to be sent to the provider.
7. Documents activities in Narrative.

01.34 Monitoring and Re-evaluation

The **Case Manager** is required to make monthly contacts with the participant. Quarterly face-to-face contacts must be made to assess the participant's current condition, needs, and services they are receiving. A face-to-face contact with a complete re-evaluation and service plan review and update is **required every 365 days**. Refer to the visit requirements outlined in the CLTC Community Choices Manual (5.37). Upon re-evaluation, another HIV Physician's Information Form does not need to be obtained. However, the CD4 count and HIV related diagnoses should be confirmed with professional(s), if possible, and documented. The "at risk for hospitalization" must be confirmed with the physician's, physician's assistant, or nurse practitioner's office. If the information is obtained from an office staff member other than the physician, nurse practitioner or physician's assistant, the documentation of the contact must include the full name and title of the person providing the information, and the full name and title of the

physician, nurse practitioner or physician's assistant from whom the information is being obtained.

If at Re-evaluation, the assessment is completed, including required information from the physician or physician's office, a level of care will be recommended. If the level of care is determined to be medically ineligible upon re-evaluation due to the CD4 count being over 500, the **Case Manager** may request a level of care exception. The following information should be provided to Central Office Designee for the level of care exception determination:

- Whether or not a previous exception has been made
- Diagnosis: HIV/AIDS or HIV related conditions
- Most recent CD4 count
- History of CD4 count
- Certification by a physician, nurse practitioner, or physician's assistant
- If the applicant/participant is "At Risk for Hospitalization"
- Service Needs
- Other resources being utilized.

01.35 Documentation

All significant contacts should be recorded in the Narrative (including checklists) in the participant's chart. Monthly contact, quarterly contact, and 365-day re-evaluation documentation should address any changes in the participant's condition or needs, the appropriateness of the service plan, and verify that all providers included in the service plan are delivering the amount and level of services that were committed.

At **re-evaluation and quarterly**, Medicaid eligibility should be documented. At **re-evaluation**, location choice and continued program participation should also be discussed and documented.

01.36 Termination

The same procedure established in the CLTC Community Choices Waiver Manual (10.2) will be used for termination.